

Claims Billing and Coding Guidelines 2026

Effective January 1, 2026 | Confidential · For Internal Use

1. General Billing Requirements

All claims must be submitted within 90 days of the date of service for commercial plans and within 365 days for Medicare Advantage plans. Claims submitted after the timely filing deadline will be denied without appeal rights. Clean claims submitted electronically will be processed within 30 days. Paper claims will be processed within 45 days.

2. Office Visit Coding · Evaluation and Management

Office and outpatient evaluation and management (E/M) services are coded using CPT codes 99202-99215. New patient visits use codes 99202-99205. Established patient visits use codes 99211-99215. Since January 1, 2021, E/M code selection is based on medical decision making (MDM) complexity or total time. Documentation must support the level of service billed.

3. CPT 99213 Documentation Requirements

CPT code 99213 (Office/outpatient visit, established patient, moderate MDM) requires documentation of at least two of the following: medical history, examination findings, or data review. Medical necessity documentation is required when CPT 99213 is billed more than 4 times in a rolling 90-day period for the same diagnosis code. Excessive billing without documentation triggers clinical review.

4. Diagnosis Code Requirements

All claims must include at least one ICD-10-CM diagnosis code at the highest level of specificity. Unspecified codes should be avoided when a more specific code is available. Claims with Z-codes as primary diagnosis are subject to additional review. Diagnosis codes must support the medical necessity of all services billed.

5. Modifiers

Modifier 25: Significant, separately identifiable E/M service on same day as procedure. Modifier 59: Distinct procedural service. Modifier 76: Repeat procedure. Modifier GT: Telehealth via interactive audio-video. Modifier 93: Synchronous telemedicine via telephone only. Incorrect modifier use may result in claim denials or overpayment recovery.

6. Laboratory and Diagnostic Services

Laboratory claims must include ordering provider NPI. Comprehensive metabolic panels (CPT 80053) require medical necessity documentation when ordered more frequently than once per 90 days without a change in clinical condition. Duplicate lab billing within the same encounter will be rejected. Reference laboratory

services require appropriate chain-of-custody documentation.

7. Coordination of Benefits

When a member has dual coverage, the primary payer must be billed first.

Coordination of benefits (COB) rules determine primary versus secondary payer.

Medicare is primary for Medicare Advantage members except where employer group health plan (EGHP) rules apply. COB disputes must be resolved before claim payment is finalized.

8. Claim Denials and Appeals

Denied claims may be appealed within 180 days of denial date. First-level appeal is reviewed by the payer's internal appeal committee. Second-level appeal includes external independent review organization (IRO). Members have the right to request an expedited appeal for urgent situations within 72 hours.